

Box 61.1 Situations in Which it is Appropriate to Consider Withdrawal From Dialysis^{1,20}

1. Patients with decision-making capacity who, being fully informed and making voluntary choices, refuse dialysis or request that dialysis be discontinued.
2. Patients who no longer possess decision-making capacity who have previously indicated refusal of dialysis through appropriate advance care planning.
3. Patients who no longer possess decision-making capacity and whose properly appointed legal agents/surrogates refuse dialysis or request that it be discontinued.
4. Patients with irreversible, profound neurologic impairment such that they lack signs of thought, sensation, purposeful behavior, and awareness of self and environment.

Box_61_1

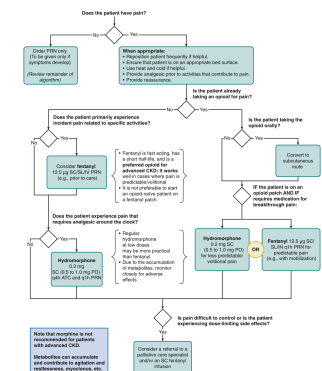


Fig 61 3

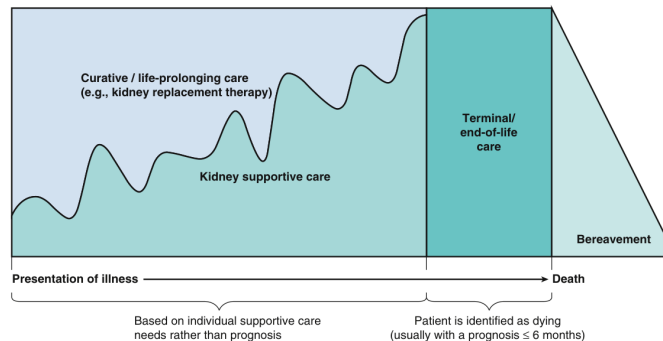


Fig. 61.1 Conceptual framework for kidney supportive care.

Fig_61_1

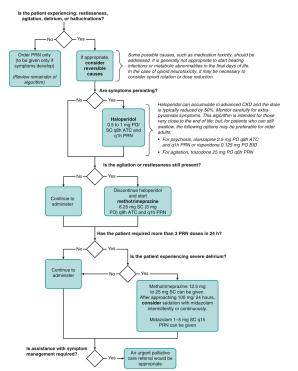


Fig 61 4

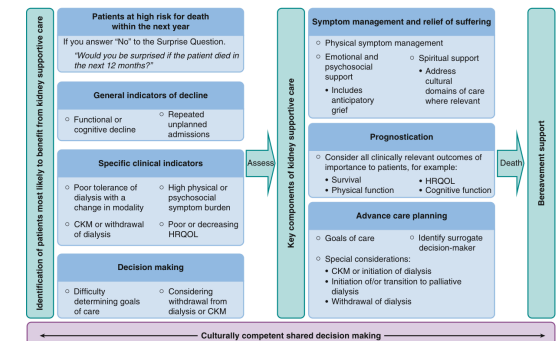


Fig. 61.2 An approach to kidney supportive care. CKM, Conservative kidney management; HRQOL, health-related quality of life. (Adapted from Davison SN. Integrating palliative care for patients with advanced chronic kidney disease: recent advances, remaining challenges. *J Palliat Care*. 2011;27:53-61.)

Fig_61_2

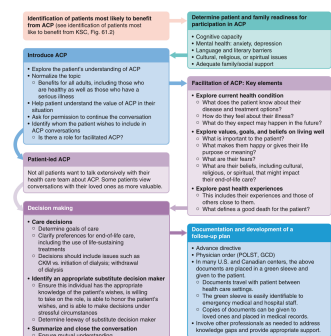


Fig. 61.5 Framework for facilitating advanced care planning (ACP) in patients with advanced chronic kidney disease. CRM, Conservative kidney management; EOL, end-of-life; GCD, goals of care designation; KSG, kidney supportive care; POLST, patient orders for life-sustaining treatment. [Adapted from Dawson SN. Facilitating advance care planning for patients with end-stage renal disease: the patient perspective. Clin J Am Soc Nephrol. 2006;1:1023-1028; Dawson SN, Torregrossa D. The creation of an advance care planning process for patients with ESRD. Am J Kidney Dis. 2007;49:27-36.]

Fig 61 5

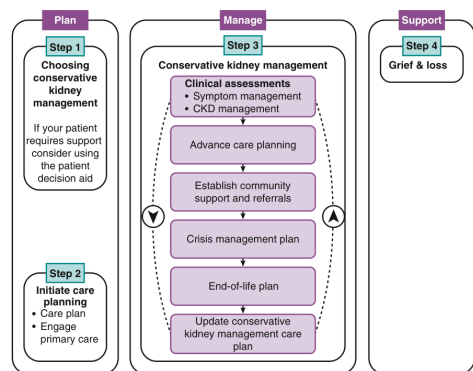


Fig. 61.6 An integrated approach to the care of people receiving conservative kidney management.^{77,92} CKD, chronic kidney disease. (Reproduced with permission from Davison SN, Kidney Supportive Care Research Group. Conservative kidney management care pathway. <<https://www.ckmcare.com>>; 2016. Accessed 26.06.23.)

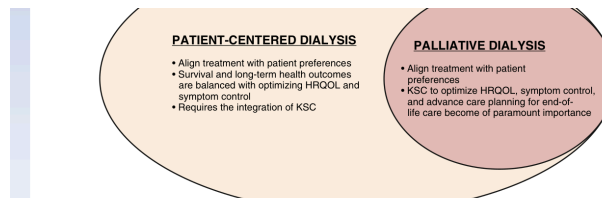


Fig. 61.7 Conceptualizing palliative dialysis. HRQOL, Health-related quality of life; KSC, kidney supportive care. (Adapted from Davison SN, Jassal SV. Supportive care: integration of patient-centered kidney care to manage symptoms and geriatric syndromes. *Clin J Am Soc Nephrol*. 2016;11:1882-1891.)

Fig_61_7

Table 6.1.1 Strategies for Culturally Competent Shared Decision Making			
Task		Communication Strategy	Additional Considerations
Understand one's own beliefs, values, and experiences		Highlighting the cultural differences from that of the patient is not always the most respectful and nonjudged strategies. It also helps to avoid conflict when a person hears that patient's beliefs and wishes are not.	Consider the influence of the health care system and institutions within which one delivers care.
		Ask about the patient's experience they have from, and degree of integration with the health care community. Assess how the patient understands it (or not) in relation to the social and cultural conditions (i.e., the cause and impact).	
Acknowledge individual cultural traditions		Ask about the patient's beliefs, values, and cultural communication activities. This includes attitudes toward health being. Avoid generalizations a patient's beliefs or values on the basis of cultural tradition.	This includes asking what the most important concerns are now they have these issues. This facilitates the practitioner for negotiating mutually acceptable goals for care.
		Ask the patient's understanding of the health care system and what they want the information told.	
Giving information		Ask/Ask this strategy can be used to many of the kinds of information he/she needs. Ask the patient's understanding of the health care system and what they want the information told.	The first task allows for a patient's health care needs to be understood and invites the patient to share concerns or explore lingering questions. Are there language barriers and/or low health literacy skills that may hinder understanding and shared decision making?
		Tell (give) information actively, avoid jargon, and disclose only one to three pieces of information at any given time.	
Determine the level of patient engagement in decision making		Ask what the patient understands or will take away from the conversation.	Explicitly explore preferences for family-oriented versus individual-centered decision making.
		Assess preferences for decision making and what will be involved.	
Address trust concerns		Be nonjudgmental, transparent, and avoid defensiveness.	This may be supported by language, level of education, socioeconomic status, and other social influences.
		Ask what tangible resources they need to take action on health care issues and assistance to allow them to and their community.	

Table 61_1

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[illegible]

Table_61_2

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Table_61_3

[illegible]

Table_61_4_part1

Table 4.1 System Management for People with Advanced Chronic Kidney Disease ^{1,2,3,4,5,6,7,8,9,10,11,12,13,14,15,16,17,18,19,20,21,22,23,24,25,26,27,28,29,30,31,32,33,34,35,36,37,38,39,40,41,42,43,44,45,46,47,48,49,50,51,52,53,54,55,56,57,58,59,60,61,62,63,64,65,66,67,68,69,70,71,72,73,74,75,76,77,78,79,80,81,82,83,84,85,86,87,88,89,90,91,92,93,94,95,96,97,98,99,100,101,102,103,104,105,106,107,108,109,110,111,112,113,114,115,116,117,118,119,120,121,122,123,124,125,126,127,128,129,130,131,132,133,134,135,136,137,138,139,140,141,142,143,144,145,146,147,148,149,150,151,152,153,154,155,156,157,158,159,160,161,162,163,164,165,166,167,168,169,170,171,172,173,174,175,176,177,178,179,180,181,182,183,184,185,186,187,188,189,190,191,192,193,194,195,196,197,198,199,200,201,202,203,204,205,206,207,208,209,210,211,212,213,214,215,216,217,218,219,220,221,222,223,224,225,226,227,228,229,230,231,232,233,234,235,236,237,238,239,240,241,242,243,244,245,246,247,248,249,250,251,252,253,254,255,256,257,258,259,260,261,262,263,264,265,266,267,268,269,270,271,272,273,274,275,276,277,278,279,280,281,282,283,284,285,286,287,288,289,290,291,292,293,294,295,296,297,298,299,300,301,302,303,304,305,306,307,308,309,310,311,312,313,314,315,316,317,318,319,320,321,322,323,324,325,326,327,328,329,330,331,332,333,334,335,336,337,338,339,340,341,342,343,344,345,346,347,348,349,350,351,352,353,354,355,356,357,358,359,360,361,362,363,364,365,366,367,368,369,370,371,372,373,374,375,376,377,378,379,380,381,382,383,384,385,386,387,388,389,390,391,392,393,394,395,396,397,398,399,400,401,402,403,404,405,406,407,408,409,410,411,412,413,414,415,416,417,418,419,420,421,422,423,424,425,426,427,428,429,430,431,432,433,434,435,436,437,438,439,440,441,442,443,444,445,446,447,448,449,450,451,452,453,454,455,456,457,458,459,460,461,462,463,464,465,466,467,468,469,470,471,472,473,474,475,476,477,478,479,480,481,482,483,484,485,486,487,488,489,490,491,492,493,494,495,496,497,498,499,500,501,502,503,504,505,506,507,508,509,510,511,512,513,514,515,516,517,518,519,520,521,522,523,524,525,526,527,528,529,530,531,532,533,534,535,536,537,538,539,540,541,542,543,544,545,546,547,548,549,550,551,552,553,554,555,556,557,558,559,560,561,562,563,564,565,566,567,568,569,570,571,572,573,574,575,576,577,578,579,580,581,582,583,584,585,586,587,588,589,590,591,592,593,594,595,596,597,598,599,600,601,602,603,604,605,606,607,608,609,610,611,612,613,614,615,616,617,618,619,620,621,622,623,624,625,626,627,628,629,630,631,632,633,634,635,636,637,638,639,640,641,642,643,644,645,646,647,648,649,650,651,652,653,654,655,656,657,658,659,660,661,662,663,664,665,666,667,668,669,670,671,672,673,674,675,676,677,678,679,680,681,682,683,684,685,686,687,688,689,690,691,692,693,694,695,696,697,698,699,700,701,702,703,704,705,706,707,708,709,710,711,712,713,714,715,716,717,718,719,720,721,722,723,724,725,726,727,728,729,730,731,732,733,734,735,736,737,738,739,740,741,742,743,744,745,746,747,748,749,750,751,752,753,754,755,756,757,758,759,760,761,762,763,764,765,766,767,768,769,770,771,772,773,774,775,776,777,778,779,780,781,782,783,784,785,786,787,788,789,790,791,792,793,794,795,796,797,798,799,800,801,802,803,804,805,806,807,808,809,810,811,812,813,814,815,816,817,818,819,820,821,822,823,824,825,826,827,828,829,830,831,832,833,834,835,836,837,838,839,840,841,842,843,844,845,846,847,848,849,850,851,852,853,854,855,856,857,858,859,860,861,862,863,864,865,866,867,868,869,870,871,872,873,874,875,876,877,878,879,880,881,882,883,884,885,886,887,888,889,890,891,892,893,894,895,896,897,898,899,900,901,902,903,904,905,906,907,908,909,910,911,912,913,914,915,916,917,918,919,920,921,922,923,924,925,926,927,928,929,930,931,932,933,934,935,936,937,938,939,940,941,942,943,944,945,946,947,948,949,950,951,952,953,954,955,956,957,958,959,960,961,962,963,964,965,966,967,968,969,970,971,972,973,974,975,976,977,978,979,980,981,982,983,984,985,986,987,988,989,990,991,992,993,994,995,996,997,998,999,1000}			
Goals and Nursing	Pharmacologic Management	Pharmacologic Management	Additional Considerations
Acidic pH Compensatory Mechanisms Metabolic Disturbances (e.g., uremic) Medications (e.g., sodium bicarbonate, sodium citrate, sodium acetate) Gastrointestinal Disturbances Compensation and decompensation and nursing interventions	Metabolic Management Monitor compensation mechanisms Monitor metabolic disturbances Monitor acid-base status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid status Monitor acid 		

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Table_61_4_part3

Table 51.5 Five Principles of Pain Management in Advanced Chronic Kidney Disease (CKD)	
Principle	Description
"By mouth"	Oral administration is the safest and therefore preferred. Patient comfort and effectiveness must be considered. If ingestion or absorption is uncertain, analgesics need to be given by alternative routes, such as transdermal, rectal, or intravenous. "By the clock" For continuous or predictable pain, analgesics should be given regularly. Additional "breakthrough" or "rescue" medication should be available on an "as needed" (PRN) basis in addition to the regular dose. "By the ladder" Pharmacologic management proceeds stepwise from a nonopioid step 1 to a very low dose of an opioid step 2 using a modified WHO analgesic ladder. There are no good options for "weak" opioid patients with advanced CKD. "For the individual" There is large variability in patients' response to analgesics. The "correct" dose is the amount needed to relieve the pain without producing intolerable side effects. Evaluation and recording of benefits and toxicity are essential. "Attention to detail" Pain changes over time; therefore there is the need for ongoing reassessment. Side effects should be explained and managed actively (e.g., constipation).
	Specific Considerations in Advanced CKD Hemodialysis patients have easy intravenous access. However, this is to be avoided as the route of administration for analgesics to optimize safety and efficacy of abuse and dependence. Some patients with mild pain may achieve adequate pain relief with analgesics doses of 25 mg or less. However, this example would be met neurophasic pain treated with gabapentin or tramadol. Hemodialysis. Continuous administration for each step of the ladder, taking into account the degree of kidney failure, is critical (Table 51.6). Sustained-release preparations are generally not recommended in people with advanced CKD. Chronic pain is often experienced in the setting of numerous other physical, psychosocial, and spiritual concerns including end-of-life issues. Opioid attention to these other issues must not be forgotten as part of the pain management strategy. There are no studies on the long-term use of analgesics in people with CKD. Careful attention must be paid to safety and efficacy. The impact on overall symptom burden, functional status, emotional state, cognition, and HRQoL should be assessed routinely.

Table_61_5

Table 61.6 Categories of Pain

	Neuropathic Pain	Nociceptive Pain
Description	Pain that results from damage to the nervous system resulting in either dysfunction or pathologic change	Pain that results from tissue damage in the skin, muscle, and other tissues, causing stimulation of sensory receptors
Pain characteristics	Common descriptors include burning, tingling, and shooting Pain may be associated with episodes of spontaneous pain, hyperalgesia, and allodynia. The presence of the latter is pathognomonic. The pain may be felt at a site distant from its cause, in the distribution of a nerve, for example	Pain may be described as sharp, like a knife – often felt at the site of damage Pain may also be experienced as dull, aching, and poorly localized (as with stimulation of visceral nociceptors)
Response to analgesics	Generally responds poorly to analgesics and requires the use of adjuvant therapy	Generally responds well to analgesics

Table_61_6

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Table_61_7